



Business Plan

A live referral profile for behavioral-health BD teams — not a patient directory.

ENTITY

CenterLinked Inc.

DATE

August 22, 2026

PRODUCT

www.centerlinked.com

LOCATION

Florida

GOVERNING LAW

Florida

STATUS

Production · early access

Document snapshot

Classification	Confidential — internal and invited advisors only
Prepared for	Founder / operating use; not a fundraising deck
Location / governing law	Florida (Terms of Service §13, effective August 22, 2026)
Source of facts	Live product, current Terms, and <code>src/lib/pricing.ts</code>

Operating numbers (customers, ARR, conversion, burn, headcount) are not in the product repository and are **not invented here**. Fill those from Stripe, admin tools, and the founder's books before using this document with investors or lenders.

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1. Executive summary

CenterLinked is a **B2B professional referral platform** for behavioral-health and addiction-treatment organizations. It replaces stale PDFs, brochures, and business cards with **one live, branded organization link** that referral partners can reopen: who the org is, where facilities are, what they treat, which insurance they are in-network with, and who to contact.

The people who **maintain** the product are BD, admissions, and marketing teams. The people who **reopen the link** are hospitals, discharge planners, case managers, therapists, other BD reps, and probation officers.

Why this exists: treatment orgs lose referrals when partners work from outdated one-pagers. Insurance networks change, BD contacts rotate, locations open, and levels of care shift. A PDF does not.

The job to be done: when a professional is ready to place a client, they need four facts fast — level of care, insurance fit, location, and a real person to call — and they need those facts to still be true next month.

Commercial model: membership is priced by **live facilities**, not seats. Referral partners who only view a public profile are not billed. Search placement is not for sale. Team seats are unlimited on every paid plan so the whole BD team shares one link.

Plan	Live facilities	Monthly	Annual (2 months free)	Done For You
Profile	1	\$99	\$990	\$499
Network	2–5	\$249	\$2,490	\$1,200
Group	6–15	\$499	\$4,990	\$2,500
Enterprise	16+	Custom (floor ~\$799/mo)	Custom	Quote

Phase 1 is shipped. Public org and program sheets, authenticated Search, facility and insurance truth, monthly verification, org dashboard, branding, team invites, referral network, downloadable org/facility one-pagers, Stripe checkout and portal, and super-admin curation are live at centerlinked.com. Community Feed and Messenger exist in the codebase and are **gated off**.

What we are not: a consumer treatment directory, a patient portal, a PHI store, a lead-gen marketplace, or a social network as the core product.

Honest status: product-complete, commercially early. Billing is live and soft-gated — inactive orgs see a banner; Search and Facilities are not hard-locked. Facility-band copy is live; the app does not yet block adding facilities above the subscribed band.

2. Company overview

Legal name	CenterLinked Inc.
Location	Florida
Governing law	Florida (Terms of Service §13)
Domain	centerlinked.com
Category	Vertical SaaS / professional network infrastructure for behavioral-health BD
Phase	Phase 1 shipped
Access model	Invite-oriented, work-email gated
Billing posture	Soft-gated early access; Stripe membership and Done For You live

Mission: make referral handoffs more trustworthy, easier to reopen, and safer to operate.

How the company earns the right to exist: BD teams already spend money on collateral that decays. CenterLinked is the persistent URL that replaces that collateral, plus the monthly verification habit that keeps it trustworthy.

How a customer uses it

1. **Claim the organization.** Request access, create a work-email account, and claim or create the treatment organization so the team has a home base.
2. **Build the profile.** Add locations, insurance contracts, levels of care, contacts, photos, and how to refer — once, in the dashboard.
3. **Share the link.** BD, admissions, and marketing send one URL (and can download a one-pager that still points back to that URL). Every reopen shows current information.

3. The problem

Behavioral-health BD is relationship-driven. Placement decisions are not “rehab near me.” They are professional judgments that need four facts fast: **level of care, insurance fit, location, and a real person to call.**

Today those facts live in the wrong artifacts:

Where the facts live today	Why it fails
PDFs, brochures, business cards	Go stale the week a payer drops, a BD rep leaves, or a location opens
Org marketing websites	Built for patients and families; named in-network payers are buried or absent
Consumer directories	Wrong audience; vague on named contracts; optimized for consumer SEO
Internal CRM	Pipeline for the selling org — does not show <i>partners</i> who you are

The cost is missed and misrouted referrals. A discharge planner who cannot confirm in-network status, or who calls a disconnected BD cell, sends the client somewhere else.

CenterLinked does **not** guarantee placements. It reduces friction so the right partner can confirm fit and reach the right contact.

4. Solution and product

One persistent URL per organization, plus per-program sheets. Update the dashboard once; every partner who reopens the link sees current information.

4.1 The public leave-behind

- **Organization sheet** at `/o/:slug` and `/:slug` — branded profile of the org, approved non-frozen facilities, how to refer.
- **Program / facility sheet** at `/o/:org/p/:program` — a single location or program. Frozen programs return not found.

- **Social preview:** OG HTML rewritten for crawlers so a shared link looks like the org, not a generic app card.
- **Downloadable one-pagers:** org overview and facility one-pagers generated from the live sheet — a conference leave-behind that still points at the live URL.

Public sheets honor `hidden_from_org_page` and only surface approved, non-frozen facilities.

4.2 Trust layer — monthly verification

Tier	Window	Meaning
Fresh	≤ 30 days	Verified this month
Recent	31–60 days	Still usable; aging
Stale	61–90 days	Needs confirmation
Frozen	> 90 days (or flagged)	Drops out of Search; program sheets return not found

Verification is the product's honesty mechanism. Catalog growth that outruns verification is a company risk, not a vanity metric.

4.3 Authenticated product

- Organization setup, create, claim, and domain-based join
- Dashboard: profile, engagement (sheet views / contact clicks), shared links
- Multi-facility management: photos, BD contacts, levels of care, specializations
- Insurance contracts per facility, linked to a curated payer database
- In-app **Search** by insurance, state, city, and level of care (approved + not frozen)
- **Referral network** — preferred partner orgs, surfaced in Search
- Team members and email invites (unlimited seats)
- Monthly contract verification workflow
- PDF facility upload + parse review (Supabase Edge Functions)
- Org branding: logo, colors, cover/footer images, social links, CTAs
- Stripe billing: facility-banded membership, annual option, Done For You, Customer Portal

4.4 Super-admin

Access requests, personal-email allowlist, organization claims, join requests, facility verifications queue, payer database, org workspace create.

4.5 Explicitly out of Phase 1

Community **Feed** and **Messenger**. UI and tables exist. `FEATURES.community === false`. Routes redirect to `/app`. Do not turn this on until sheets + Search are clearly the habit.

5. Market

Industry backdrop (published estimates, **not** CenterLinked data): U.S. mental health and addiction treatment is a large, fragmented market. IBISWorld counts on the order of **5,128 residential mental-health / substance-abuse businesses** (2026). Licensed SUD facilities have been estimated around **17,000**. Those figures describe the industry we sell into, not software TAM, and they mix facilities with organizations.

Bottoms-up software TAM: U.S. behavioral-health organizations that take **professional** referrals and would pay to keep a live profile current. A defensible org-count is on the order of **5,000–15,000** (many facilities sit under one org). This is a focused vertical, not a horizontal “all healthcare” TAM.

At facility-banded pricing, ARPU is no longer a flat \$1,188/year:

Mix of 400 paying orgs	Illustrative ARR
All Profile (\$99/mo)	~\$475k
50% Profile / 35% Network / 15% Group	~\$1.07M

These are **scenarios, not forecasts**. They exist to show that a cheap Profile on-ramp can still build catalog density while the ICP (multi-site groups) pays like multi-site groups.

What we are not sizing: consumer “rehab near me” search, patient-pay lead gen, or national directory advertising. Winning that market would require becoming a different company.

6. Customers

Who pays

The **organization**. Checkout is restricted to `facility_admin` or `super_admin`. Typical buyers: org leadership, BD leadership, admissions / marketing.

Who operates

BD reps, admissions, marketing. Work-email only. **Seats are unlimited** on every plan — charging per rep would punish the behavior we want (the whole team sharing one link).

Who consumes the link (does not pay)

Hospitals, therapists, case managers, other BD reps, probation officers, discharge planners. They are the reason the link exists. They are never the invoice.

ICP

Multi-location treatment orgs with an active BD team — regional groups (Network) and multi-site platforms (Group / Enterprise). They already print collateral, already work conferences, and already lose deals to stale insurance lists.

Anti-ICP

Consumer SEO shops and anyone who wants CenterLinked to “get us patients from Google.” That buyer will churn or demand features that destroy the brand.

Role	Meaning
super_admin	Platform operator
facility_admin	Org admin — profile, facilities, billing, members
bd_rep	Standard org member (default invite / join)

UI gates are not security. RLS and SECURITY DEFINER RPCs are.

7. Competitive landscape

Alternative	Why CenterLinked is different
PDFs / brochures / cards	Go stale; version chaos across every BD inbox
Org website	Wrong audience; insurance buried; not built as a leave-behind
Rehabs.com, Recovery.com, Psychology Today	Consumer directories; paid listings; not a professional source of truth
CRM	Internal pipeline — does not show partners who you are
Paid “featured listing” directories	CenterLinked does not sell Search placement
Generic link-in-bio / microsite tools	No payer database, no monthly freeze semantics, no work-email professional network

Positioning line: not a directory for families. A live referral profile for professionals.

There is no serious incumbent whose only job is “one verified org link for BD.” That is an advantage and a risk: the category has to be explained, not captured.

8. Business model

Who pays: the organization. Never the referral partner. Never a pay-to-rank Search slot.

Membership is priced on live facilities the org maintains, in bands (not metered overage). Orgs upgrade when they outgrow a band. Everything that is the product stays in every paid plan: org + program sheets, branding, insurance, contacts, unlimited team, Search, monthly verification.

Annual billing is **two months free** (\$990 / \$2,490 / \$4,990). Enterprise (16+) is quoted, with a published floor around \$799/month.

Done For You is labor-scaled setup on top of membership. \$499 is the one-location landing number. A 10-location build is \$2,500, not \$499. 16+ is quoted. DFY sold below the labor it takes destroys margin on the best accounts.

Early access (current commercial reality)

- Inactive subscriptions see a dismissible banner. Search and Facilities are not hard-locked.
- Terms §8: this may change; notice before a new price is charged.
- Facility-band copy is live. The app does not yet block adding facilities above the subscribed band.

Unit economics (structural, not measured)

Software COGS (Supabase, Vercel, Stripe, Resend) is low relative to price. The constraint is **ops cost per org**: access review, payer curation, DFY builds, verification follow-up.

Unknowns to fill from Stripe and admin tools (do not guess): access-request → paying conversion; mix across tiers; DFY attach; annual vs monthly mix; logo retention / churn at month 6 and 12; support and DFY hours per org.

9. Go-to-market

Invite + request-access + work-email. **Not consumer SEO.**

1. **Founder-led BD** into treatment orgs you already know. Conference follow-up is the native use case — the link (and one-pager) is the leave-behind.
2. **Done For You** as the wedge for busy CEOs, now priced so a 10-location build is not \$499.
3. **Make the public link the habit**: QR, email signature, post-meeting text, one-pager handout. The URL is the product.
4. **Density in 1–2 metros** so authenticated Search is actually useful.
5. **Do not** buy “rehab near me.” **Do not** promise to fill beds.

Channels that fit: treatment conferences, existing BD relationships, warm intros, and the public sheet itself when a partner asks “who are you in-network with?”

Channels that do not fit: consumer paid search, patient lead marketplaces, open self-serve consumer signup.

10. Operations and technology

Stack: Vite 5 + React 18 SPA, TypeScript, Tailwind + shadcn/ui. Supabase (Auth, Postgres + RLS, Storage, Edge Functions). Stripe Checkout, Customer Portal, webhooks. Resend. Deployed on Vercel (SPA + api/ serverless + middleware.js for OG).

Access: work-email gated. Personal domains blocked unless listed in `approved_personal_emails` or `bootstrap_admin_emails`. Enforced client-side (`is_email_auth_allowed`) and via `/api/auth-before-user-created`.

Facility writes go through `saveFacilityWithContracts` → RPC `save_facility_with_contracts`.

Catalog item	How it is charged
Profile \$99/mo	STRIPE_PRICE_MEMBERSHIP
Profile annual \$990	STRIPE_PRICE_PROFILE_YEAR
Network / Group month + year	Dedicated Stripe Price IDs
Done For You (1 / 2-5 / 6-15)	Dedicated one-time Price IDs

Security posture that is also a sales asset: no PHI by design, work-email gate, tenant isolation via RLS. The browser never receives SUPABASE_SERVICE_ROLE or other secrets.

11. Regulatory, privacy, and brand risk

CenterLinked is an **organization profile platform**. It is not a covered entity's clinical system and must not become one.

Rule	Why it is existential
No patient records / PHI	A single PHI incident would redefine the company as a healthcare app it is not staffed or designed to be
No placement guarantee	Terms and FAQ already say this; promising beds is a legal and brand lie
Named in-network listings are for fit	Benefits verification stays in admissions
Work-email gate	Keeps the network professional and reduces consumer-directory drift
No paid Search rank	Selling placement would make the catalog untrustworthy

Wrong-but-confident expansion into patient matching or PHI would destroy the company faster than slow sales. Terms (effective August 22, 2026) and Privacy Policy are live. Fees in Terms §8 match the facility-banded catalog. Governing law and venue are **Florida**.

12. Traction

Shipped and in production: marketing landing; auth and work-email gate; access intake; org setup / claim / join; dashboard, facilities, Search, referral network, members, settings, branding; public org and program sheets + OG; org and facility one-pagers; Stripe checkout, portal, webhook idempotency; monthly verification; super-admin tools; PDF upload / parse path.

Metric	Source	Status in this plan
Orgs created vs approved vs paying	Admin + Stripe	Unknown — do not invent
Mix by Profile / Network / Group	Stripe + organizations	Unknown
Facilities live vs frozen	Admin / DB	Unknown
Public-link reopens and contact clicks	org_analytics_events	Unknown
DFY attach	Stripe	Unknown
Access-request volume and conversion	early_access_leads	Unknown
Verify-on-time rate	contracts_verified_at	Unknown

Until those are filled, describe the company as **product-complete, commercially early**.

13. 18-month operating plan

Months 0–6 — Density and habit

Tight ICP. Shareable link within days (DFY default for first cohort). Do not turn on community. Instrument the leave-behind: shares, QR, one-pager downloads, reopens.

Success: member orgs have a live link they actually send; first metro has enough approved facilities that Search is not empty.

Months 6–12 — Catalog that Search is worth opening

One or two geographies. Instrument shares, reopens, verify-on-time rate. Sell annual in Checkout (already offered). Use DFY to land multi-site groups at Network/Group, not only Profile.

Success: Search is useful for a real insurance + geography query in the focus metros; annual mix is visible in Stripe.

Months 12–18 — Enforce what we sell

Named date to hard-gate unpaid orgs (or grandfather early-access logos). Enforce facility bands on create/save if mix shows gaming. Revisit community only if sheets + Search are clearly the habit.

Success metrics (targets to set from real baselines, not invented here): % of member orgs with a fresh verification; public-link reopens per org per month; paid conversion after early access; logo retention at month 6 and 12; share of revenue from Network + Group vs Profile.

14. Team, financials, and funding

Not in the product repo. Do not send this plan to a third party with invented headcount, salaries, burn, runway, or a raise amount.

What belongs in a companion one-pager (founder-prepared, not guessed here): legal capitalization and officers; current monthly burn and runway; Stripe MRR / ARR and cash collected; whether a raise is

contemplated, and for what use of proceeds; who operates access review, payer curation, and DFY delivery.

This document is the **product and commercial plan**. It is not a substitute for financial statements.

15. Risks and mitigations

Risk	Mitigation
Slow adoption	DFY wedge; founder-led; metro density; keep \$99 Profile on-ramp
Catalog rot	Freeze semantics; do not grow faster than verification
Billing stays soft forever	Named date to hard-gate or grandfather
Facility-band gaming	Enforce caps when early access ends
Confused with consumer directories	Keep work-email gate; refuse patient features
PHI creep	Product principles; no clinical workflows
DFY labor overrun	Never sell \$499 for a 12-site build
Search empty in new geos	Density before national slogans
Community turned on too early	Flag stays off until sheets + Search are the habit
Live Stripe catalog lag	Mirror the test catalog in live Stripe and Vercel env

16. The line we will not cross

Even if a feature would be “easy”: consumer treatment directory; patient intake, scheduling, medical advice, or clinical decision support; PHI / patient records; open self-serve consumer lead generation; social network as the Phase-1 core; paid Search placement.

If a request would make CenterLinked any of the above, the answer is no.

17. Why this can work

The job is real: BD teams already spend money on collateral that decays. The product is live and narrow. Trust is designed in (work email, monthly verify, freeze). Pricing now matches the unit that creates work and value — **facilities** — while seats stay free so the whole BD team shares one link.

The honest constraint: this is a focused vertical SaaS. It wins by becoming the default leave-behind in professional placements, not by winning Google for “rehab.”

Appendix A — Pricing catalog

Source of truth: `src/lib/pricing.ts` and `server/stripe/pricing.mjs`. Must stay in sync with landing, billing UI, Terms §8, and `public/llms.txt`.

Included on every paid plan: organization dashboard · public shareable profile · unlimited team seats · unlimited profile updates · monthly verification · insurance and level of care listings · referral contact management · authenticated Search.

Plan	Facilities	Monthly	Annual	Positioning
Profile	1	\$99	\$990	One location. One live link.
Network	2–5	\$249	\$2,490	Regional groups. Featured as “Most groups.”
Group	6–15	\$499	\$4,990	Multi-site source of truth.
Enterprise	16+	Quote from ~\$799/mo	Quote	National platforms. Request access.

Done For You band	One-time amount
1 facility	\$499
2–5 facilities	\$1,200
6–15 facilities	\$2,500
16+	Quote

Referral partners who only view a public profile are not billed.

Appendix B — Shipped product surface

Surface	Status
Landing + Privacy + Terms	Live
Request access	Live
Auth (email, Google, work-email gate)	Live
Org setup / claim / join	Live
Org dashboard, facilities, insurance	Live
Monthly verification	Live
Authenticated Search + referral network	Live
Team members / invites	Live
Public org + program sheets + OG	Live
Org / facility one-pagers	Live
Stripe membership + DFY + portal	Live (soft-gated)
Super-admin tools	Live
PDF parse onboarding	Live (Edge Functions)
Community Feed / Messenger	Built, gated off

Appendix C — Open items before third-party use

Do not send this plan to an investor, bank, or large customer until the founder fills:

1. Paying org count and MRR / ARR (Stripe)
2. Tier mix and DFY attach
3. Access-request conversion
4. Facilities live vs frozen; verify-on-time rate
5. Public-link reopens per org
6. Team, burn, runway
7. Named date (or grandfather policy) for ending soft-gated billing
8. Mirror the Stripe catalog and price IDs into **live** Stripe and Vercel

Appendix D — Sources and notes

- Product facts: live app at <https://www.centerlinked.com>, PROJECT.md, PRINCIPLES.md, ARCHITECTURE.md
- Pricing: `src/lib/pricing.ts`, Terms of Service §8 (effective August 22, 2026)

- Verification windows: src/lib/verification.ts
- Industry backdrop: IBISWorld residential MH/SA business count (2026) and published licensed-SUD facility estimates — industry context only, not CenterLinked TAM
- This plan does not invent customers, revenue, conversion, burn, or headcount

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